

August 20, 2026

Aetna
Provider Resolution Team
P.O. Box 14020
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Marc Corkery (MRN MUSC8042994), Claim MUSC-76E3-4

Patient	Marc Corkery (DOB 1977-08-10)
MRN	MUSC8042994
Member ID / Group	AET942376912 / GRP-91373
Claim number	MUSC-76E3-4
Date of service	2026-04-18
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	R69 — Illness, unspecified
Denied amount	\$570.04
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-11-11

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this Level 1 reconsideration on behalf of Marc Corkery (MRN: MUSC8042994, Member ID: AET942376912, Group: GRP-91373) regarding claim MUSC-76E3-4 for a date of service of April 18, 2026, billed by rendering provider James H. Ferrell, MD (NPI 1992837465) at MUSC Health East Cooper Medical Center. The claim was denied on May 15, 2026, under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying payer remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." The total denied amount is \$570.04 for one unit of CPT 99214, an established patient office visit of moderate complexity. MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the allowed amount of \$381.70.

CLINICAL BACKGROUND

Mr. Corkery is a 49-year-old male with an active diagnosis of obesity (ICD-10 E66.9), documented as ongoing since 2015, with a recorded body mass index exceeding 30 kg/m² and body weight of approximately 103.5 kg on prior encounters. He also carries an active prescription regimen that includes a fluticasone propionate metered dose inhaler and an albuterol metered dose inhaler, both of which reflect a chronic respiratory management need requiring ongoing clinical oversight. His medical history includes prior episodes of acute bronchitis and a resolved second-degree burn, for which analgesic and antibiotic therapy was previously prescribed. The April 18, 2026, encounter was billed under primary diagnosis R69 (Illness, unspecified) with a secondary diagnosis of E66.9 (Obesity, unspecified). The visit was rendered in an on-campus outpatient hospital setting (Place of Service 22) and coded at the moderate-complexity level consistent with CPT 99214.

BASIS FOR APPEAL

Aetna's denial rests on the assertion that the documentation does not establish medical necessity for the service as reported. MUSC Health contends that this determination is inconsistent with the clinical facts of record. Mr. Corkery presents with active obesity and requires ongoing management of chronic respiratory conditions evidenced by his active inhaler regimen. An established patient visit with a physician to monitor and manage these active, documented conditions is consistent with generally accepted standards of medical practice and satisfies the threshold for medical necessity under any reasonable clinical review standard.

The selection of CPT 99214 reflects a moderate-complexity evaluation and management service, which is appropriate given the patient's active chronic conditions, ongoing pharmacologic management, and the clinical judgment required to oversee his care. The use of R69 as the primary diagnosis code, while broad, does not negate the medical necessity of the visit when the totality of the patient's documented conditions and active medications is considered. Payers are expected to evaluate medical necessity based on the complete clinical picture, not solely on the specificity of a single diagnosis code. MUSC Health is prepared to submit the complete encounter documentation, including the physician's notes, to support the complexity and necessity of this visit.

Furthermore, denial based on an LCD reference requires that Aetna identify the specific coverage determination implicated and demonstrate that the service falls outside its scope. To the extent the denial is predicated on diagnosis code specificity rather than a genuine absence of clinical need, that basis does not constitute a valid medical necessity denial under standard coverage principles.

REQUESTED ACTION

MUSC Health respectfully requests that Aetna conduct a full clinical review of the submitted documentation and reverse the denial of claim MUSC-76E3-4. We request payment of the allowed amount of \$381.70 for CPT 99214 rendered on April 18, 2026. If additional clinical documentation is required to complete this review, please contact MUSC Health Revenue Cycle at your earliest convenience so that records may be provided promptly and within the applicable appeal timeframe of November 11, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record